

Telemedicine Consult Guidelines & Process

Standard:

To establish guidelines and process for all System Credentialed participants and providers when utilizing telemedicine as part of patient care.

Purpose:

Primary goal is to provide expanded service to our underserved and vulnerable populations, relieving pressure on field crews, and keeping more ambulances available for higher acuity calls.

Patient Selection:

1. Patient age $\geq$ 6 months old; and
2. No history of coronary artery disease (ie. Heart attack, coronary artery bypass); and
3. No acute hypotensive, syncope, or near syncope event(s); and
4. No episodes of hypoxia while in EMS care
5. Can ambulate, function, and perform Activities of Daily Living (ADLs) at baseline; and
6. Adult vital signs are within the following parameters (Pediatric vital signs within normal ranges):
 - a. Pulse: 60 – 110 bpm
 - b. Systolic BP: 90 – 200 mmHg
 - c. Respirations: 12 – 30 bpm
 - d. Blood Glucose: 70 – 300 mg/dL; no signs of DKA
 - e. SPO₂ $\geq$ 93% on room air or prescribed supplemental oxygen

If patient does not meet above criteria but provider believes that patient would benefit from telemedicine, provider may contact OLMC

Field Providers:

1. Review Patient Selection criteria.
2. Determine the most appropriate clinical guideline(s) based on patient complaint and presentation.
3. Perform complete patient assessment, physical exam, and diagnostics as informed by applicable clinical guideline(s).
4. Obtain complete vital signs, including EtCO₂ when indicated by patient complaint, presentation, or applicable clinical guideline(s).
5. Use Pulsara, goal is to include a picture.
6. Lead provider contacts telemedicine and use standardized reporting format.
7. Assign the telehealth provider to your EHR.
8. Document telehealth interaction in addition to standard patient care and narrative.

C4:

1. Screen for Patient Selection criteria.
2. Determine the most appropriate clinical guideline(s) based on patient complaint then review the indicated assessment, exam, and diagnostics with field providers.
3. All telemedicine calls are made using Pulsara.
4. Use standard format with brief intro to telemedicine provider, majority of findings and patient information needs to be communicated from the lead field provider to the telehealth provider.
5. Document interaction and consult.
6. Send discharge instructions to patients when able.
7. Follow up / Call back as directed.

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Telehealth Provider:

1. Complete telemedicine consult using video and Pulsara.
2. Document telemedicine consult in EHR(s).